Name	:	Ms. TANU SEHGAL			
Lab No.	:	190397913	Age	:	54 Years
Ref By	:	Self	Gender	:	Female
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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
SWASTHFIT COMPLETE PACKAGE - NEW			
SUGAR CHOICE, PLASMA (Hexokinase)			
Glucose, Fasting	84.00	mg/dL	70.00 - 100.00
IRON STUDIES, SERUM			
Iron (Ferrozine)	97.00	µg/dL	50.00 - 170.00
Total Iron Binding Capacity (TIBC) (Chromozural B)	315.00	µg/dL	250 - 425
Transferrin Saturation (Calculated)	30.79	%	15.00 - 50.00

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIPID PROFILE BASIC			
Cholesterol Total (CHO-POD)	188	mg/dL	<200
Triglycerides (GPO-POD)	98	mg/dL	<150
HDL Cholesterol (Enz Immunoinhibition)	60	mg/dL	>50
LDL Cholesterol,Direct (Enz Selective protection)	**118**	mg/dL	<100
VLDL Cholesterol (Calculated)	20	mg/dL	<30
Non-HDL Cholesterol (Calculated)	128	mg/dL	<130

Note

1. Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
2. Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors
3. Triglycerides levels >150 mg/dL in fasting or >175 mg/dL in non-fasting are considered risk modifier for ASCVD risk
4. Test conducted in Serum

Treatment Goals for Lipid lowering therapy*

ASCVD RISK CATEGORY	TREATMENT GOAL: LDL-C in mg/dL (Primary target)	TREATMENT GOAL: NON HDL-C in mg/dL (Co-Primary target)
Low	<100	<130
Moderate	<100	<130
High	<70	<100
Very High	<50	<80
Extreme (A)	<50 (<30 optional)	<80 (< 60 optional)
Extreme (B)	<30	<60

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval

ASCVD Risk Stratification in Indian population*

Indians are at very high risk of developing ASCVD, they usually get the disease at an early age, have a more severe form of the disease and have poorer outcome as compared to the western populations. Many individuals remain asymptomatic before they get heart attack, ASCVD risk helps to identify high risk individuals even when there is no symptom related to heart disease. Risk stratification is important to guide lipid lowering therapy and to identify treatment goals.

**Reference: Lipid Association of India 2023 update on cardiovascular risk assessment and lipid management in Indian patients: Consensus statement IV*

Please Note: *A more formal risk assessment may be used by clinicians according to their personal preferences and familiarity with the risk scores*

Risk factors/markers

Major ASCVD risk factors	High-risk features	Risk modifiers
1. Age ≥45 years in males and ≥55 years in females 2. Current cigarette smoking or tobacco use* 3. High blood pressure* 4. Low HDL-C	1. Family history of premature ASCVD 2. CKD stage 3B or 4 3. Apolipoprotein B >130 mg/dL 4. Extreme elevation of a single risk factor† 5. Lipoprotein (a) ≥50 mg/dL 6. Metabolic syndrome 7. Non-alcoholic fatty liver disease with fibrosis grade 2 or 3 fibrosis 8. CACS 1-99 and <75th percentile	1. Lipoprotein (a) 20-49 mg/dL 2. Impaired fasting glucose (fasting blood glucose 100-125 mg/dL)‡ 3. Increased waist circumference (>90 cm in men, >80 cm in women)§ 4. hsCRP >2 mg/L¶ 5. Plasma triglycerides >150 mg/dL fasting or >175 mg/dL non-fasting 6. Rheumatoid arthritis, psoriasis, and spondyloarthropathies 7. Premature menopause, pre-eclampsia, gestational diabetes, PCOS 8. High polygenic risk score 9. Air pollution 10. Human immunodeficiency virus infection

Risk groups

Low risk	Moderate risk	High risk	Very high risk	Extreme risk
0-1 major ASCVD risk factor, and LDL-C 100-129 mg/dL, and Non-HDL-C 130-159 mg/dL, and Life-time CVD risk <30%#	• 2 major ASCVD risk factors, or • LDL-C 130-159 mg/dL or • Non-HDL-C 160-189 mg/dL or • Low-risk group with ≥1 risk modifier or lifetime ASCVD risk >30%	• ≥3 major ASCVD risk factors, or • LDL-C 160-189 mg/dL or • Non-HDL-C 190-219 mg/dL or • Diabetes with 0-1 major ASCVD risk factors or • 2 major ASCVD risk factor + ≥1 risk modifier or • Any 1 high-risk feature	• Diabetes with target organ damage • Diabetes with ≥2 major ASCVD risk factors • CACS 100-299 or >75th percentile if CACS 1-99 • ≥2 high risk features • Established ASCVD (obstructive or non-obstructive)$ • Heterozygous FH or LDL-C ≥190 mg/dL	**Category A** • ASCVD with ≥1 feature of high-risk group • CACS ≥300 • Homozygous FH **Category B** ASCVD with- • ≥1 feature of very high-risk group, or • Recurrent ACS, or • Polyvascular disease, or • Homozygous FH **Recurrent ASCVD event despite LDL-C around 30 mg/dL** These patients require special consideration, please see the text for more details- **Category C**

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIVER & KIDNEY FUNCTION TEST			
Creatinine (Modified Jaffe,Kinetic)	0.72	mg/dL	0.55 - 1.02
GFR Estimated (CKD EPI Equation 2021)	99	mL/min/1.73m2	>59
GFR Category (KDIGO Guideline 2012)	G1		
Urea (Urease UV)	19.30	mg/dL	13.00 - 43.00
Urea Nitrogen Blood (Calculated)	9.01	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio (Calculated)	13		
Uric Acid (Uricase)	4.00	mg/dL	2.60 - 6.00
AST (SGOT) (IFCC without P5P)	29.0	U/L	13.00 - 35.00
ALT (SGPT) (IFCC without P5P)	20.0	U/L	10.00 - 49.00
AST:ALT Ratio (Calculated)	**1.45**		<1.00
GGTP (IFCC,L-y-glutamyl-3-carboxy-4-nitroanilide)	14.0	U/L	0 - 38
Alkaline Phosphatase (ALP) (IFCC-AMP)	63.00	U/L	30.00 - 120.00
Bilirubin Total (Oxidation)	0.53	mg/dL	0.30 - 1.20
Bilirubin Direct (Oxidation)	0.14	mg/dL	<0.3
Bilirubin Indirect (Calculated)	0.39	mg/dL	<1.10
Total Protein (Biuret)	7.70	g/dL	5.70 - 8.20
Albumin (BCG)	4.30	g/dL	3.20 - 4.80
Globulin(Calculated)	3.40	gm/dL	2.0 - 3.5
A : G Ratio (Calculated)	1.26		0.90 - 2.00

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
Calcium, Total (Arsenazo III)	10.20	mg/dL	8.70 - 10.40
Phosphorus (Molybdate UV)	3.90	mg/dL	2.40 - 5.10
Sodium (Indirect ISE)	143.00	mEq/L	136.00 - 145.00
Potassium (Indirect ISE)	**5.79**	mEq/L	3.50 - 5.10
Chloride (Indirect ISE)	106.00	mEq/L	98.00 - 107.00

Note: Test conducted in Serum

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
VITAMIN B12 (CLIA)			
Vitamin B12; Cyanocobalamin	**1426.00**	pg/mL	211.00 - 911.00

Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients
4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity
5. Test conducted on serum

Test Name	Results	Units	Bio. Ref. Interval
VITAMIN D, 25 - HYDROXY, SERUM (CLIA)			
Vitamin D, 25 Hydroxy	147.99	nmol/L	75.00 - 250.00

Interpretation

LEVEL	REFERENCE RANGE IN nmol/L	COMMENTS
Deficient	< 50	High risk for developing bone disease
Insufficient	50-74	Vitamin D concentration which normalizes Parathyroid hormone concentration
Sufficient	75-250	Optimal concentration for maximal health benefit
Potential intoxication	>250	High risk for toxic effects

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval

- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.
- A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available
- Test conducted on serum

SERUM AMYLASE
(G7PNP)

Test Name	Results	Units	Bio. Ref. Interval
Amylase	108.00	U/L	30.00 - 118.00

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HbA1c (HPLC,NGSP Certified)			
HbA1c	5.1	%	4.00 - 5.60
Estimated average glucose (eAG)	100	mg/dL	

Interpretation

HbA1c result is suggestive of non diabetic adults (>=18 years)/ well controlled Diabetes in a known Diabetic

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults >=18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	>= 6.5	<7.0

Note

1. Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.
2. Test conducted in EDTA Whole blood

FACTORS THAT INTERFERE WITH HbA1c MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HBA1C RESULTS
Hemoglobin variants,elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g.,recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used.Iron deficiency anemia is associated with higher HbA1c

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
Gross Examination			
Microscopy			
URINE EXAMINATION, AUTOMATED	Sample Not Received		

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
High Sensitivity CRP (Immunoturbidimetry)	**1.90**	mg/L	<1.00

Interpretation

CARDIO CRP IN mg/L	CARDIOVASCULAR RISK
<1	Low
1-3	Average
3-10	High
>10	Persistent elevation may represent Non cardiovascular inflammation

Note: Test conducted on serum

Test Name	Results	Units	Bio. Ref. Interval
APOLIPOPROTEINS A1 & B, SERUM (Immunoturbidimetry)			
Apolipoprotein (Apo A1)	127	mg/dL	76 - 214
Apolipoprotein (Apo B)	90	mg/dL	46 - 142
Apo B / Apo A1 Ratio	0.71		0.35 - 0.98

Note: Test conducted on serum

As per recommendations of National Cholesterol Education Program (NCEP) the clinical significance of results is as follows:

Apolipoprotein B

RESULT IN mg/dL	REMARKS
<23	Abetalipoproteinemia/Hypobetalipoproteinemia
23-45	Hypobetalipoproteinemia
46-135	Normal
>135	Hyperapobetalipoproteinemia/Increased CAD risk

Apo B to A1 Ratio

RATIO	REMARKS
0.35-0.98	Desirable
>0.98	Increased CAD risk

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
THYROID PROFILE FREE (CLIA)			
Free Triiodothyronine (T3, Free)	2.90	pg/mL	2.30 - 4.20
Free Thyroxine (T4, Free)	1.18	ng/dL	0.89 - 1.76
TSH, Ultrasensitive	2.305	µIU/mL	0.550 - 4.780

Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm. The variation is of the order of 50%. hence time of the day has influence on the measured serum TSH concentrations.

2. TSH Values <0.03 µIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals

3. Test conducted on serum

Reference Ranges for pregnancy

PREGNANCY	REFERENCE RANGE for TSH in µIU/mL (As per American Thyroid Association)	REFERENCE RANGE for FT3 in pg/mL	REFERENCE RANGE for FT4 in ng/dL
1st Trimester	0.100 - 2.500	2.11-3.83	0.70 -2.00
2st Trimester	0.200 - 3.000	1.96-3.38	0.50 -1.60
3st Trimester	0.300 - 3.000	1.96-3.38	0.50 -1.60

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM			
Hemoglobin (Photometry)	**11.50**	g/dL	12.00 - 15.00
Packed Cell Volume (PCV) (Calculated)	**35.40**	%	36.00 - 46.00
RBC Count (Electrical impedence)	3.92	mill/mm3	3.80 - 4.80
MCV (Electrical impedence)	90.40	fL	83.00 - 101.00
Mentzer Index (Calculated)	23.1		
MCH (Calculated)	29.30	pg	27.00 - 32.00
MCHC (Calculated)	32.40	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW) (Electrical Impedence)	**15.30**	%	11.60 - 14.00
Total Leukocyte Count (TLC) (Electrical Impedence)	5.60	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils (VCS Technology)	54.30	%	40.00 - 80.00
Lymphocytes (VCS Technology)	34.80	%	20.00 - 40.00
Monocytes (VCS Technology)	7.70	%	2.00 - 10.00
Eosinophils (VCS Technology)	2.70	%	1.00 - 6.00
Basophils (VCS Technology)	0.50	%	<2.00
Absolute Leucocyte Count			
Neutrophils (Calculated)	3.04	thou/mm3	2.00 - 7.00
Lymphocytes (Calculated)	1.95	thou/mm3	1.00 - 3.00
Monocytes (Calculated)	0.43	thou/mm3	0.20 - 1.00
Eosinophils (Calculated)	0.15	thou/mm3	0.02 - 0.50

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
Basophils (Calculated)	0.03	thou/mm3	0.02 - 0.10
Platelet Count (Electrical impedence)	212	thou/mm3	150.00 - 410.00
Mean Platelet Volume (Electrical impedence)	11.7	fL	6.5 - 12.0
E.S.R. (Capillary photometry)	**34**	mm/hr	0.00 - 30.00

Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

Note

1. As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
2. Test conducted on EDTA whole blood

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
C-REACTIVE PROTEIN (CRP) (Immunoturbidimetry)	2.00	mg/L	<3.30

Note: Test conducted on serum

DMC - 115396
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Technical Director - Hematology & Immunology
NRL - Dr Lal PathLabs Ltd

Dr.Anirudh Bharat Kumar Gupta
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DMC – 67327
Dr Anjalika Goyal
MD,Biochemistry
Consultant Biochemist
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DMC - 89819
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DMC – 9550
Dr Nimmi Kansal
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Technical Director - Clinical Chemistry & Biochemical Genetics
NRL - Dr Lal PathLabs Ltd

DMC - 24201
Dr Sarita Kumari Lal
MD, Pathology
Senior Consultant Pathologist
Dr Lal PathLabs Ltd

DMC NO.16715
Dr Shalabh Malik
MD, Microbiology
Technical Director - Microbiology, Infectious Disease Molecular & Serology, Clinical Pathology
NRL - Dr Lal PathLabs Ltd

MCI 15-19066
Dr.Richa Sirohi
MD, Biochemistry
Senior Consultant Biochemist
NRL - Dr Lal PathLabs Ltd

DMC – 46663
Dr Sunanda
MD, Pathology
Sr. Consultant Pathologist - Hematology & Immunology
NRL - Dr Lal PathLabs Ltd

-------------------------------End of report --------------------------------

Regd. Office: Dr Lal PathLabs Ltd, Block-E, Sector-18, Rohini, New Delhi-110085
Web: www.lalpathlabs.com, CIN: L74899DL1995PLC065388

Name	:	Ms. TANU SEHGAL			
Lab No.	:	190397913	Age	:	54 Years
Ref By	:	Self	Gender	:	Female
Collected	:	23/11/2025 9:15:00AM	Reported	:	23/11/2025 1:10:20PM
A/c Status	:	P	Report Status	:	Final
Collected at	:	LPL-ROHINI (NATIONAL REFERENCE LAB) National Reference laboratory, Block E, Sector 18, ROHINI DELHI 110085	Processed at	:	LPL-NATIONAL REFERENCE LAB National Reference laboratory, Block E, Sector 18, Rohini, New Delhi -110085

MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval

IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted.•All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician.•Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.•Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.•Test results may show interlaboratory variations.•The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).•Test results are not valid for medico legal purposes.•This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor.•The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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National Reference lab, Delhi, a CAP (7171001) Accredited, ISO 9001:2015 (FS60411) & ISO 27001:2013 (616691) Certified laboratory.